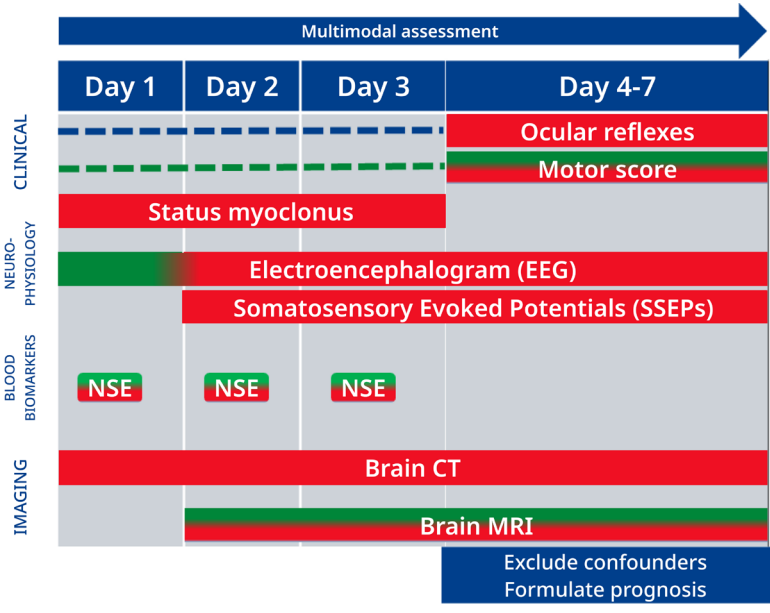
SUGGESTED TIMINGS FOR MULTIMODAL PREDICTORS AND NEUROLOGICAL PROGNOSIS

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POST RESUSCITATION CARE



Prediction of good outcome

Prediction of poor outcome

* Use short-acting agents and reduce or stop sedation before using EEG for neuroprognostication

NEUROLOGICAL PROGNOSTICATION

Unconscious patient
Motor score 5 or less at 72 h or later, and confounders excluded*

*At least **TWO** unfavourable signs:*

- No pupillary** and corneal reflexes at 72 h or later
- Bilaterally absent N20 SSEP wave at 24 h or later
- Suppression or burst-suppression on EEG at 24 h or later
- NSE higher than 60µg L^{-1} *** at 48 h and / or 72 h
- Status myoclonus**** within 72 h
- Diffuse and extensive hypoxic-ischaemic injury on brain CT at any time or on MRI after 2 days

NO

ANY *favourable sign:*

- GCS-M 4 or 5
- Continuous, normal voltage EEG background within 72 h
- NSE 17µg L^{-1} or less within 72 h
- No diffusion changes on MRI at 2-7 days

NO

Indeterminate outcome
Observe and re-evaluate

YES

Poor neurological outcome very likely

YES

Good neurological outcome likely

SCAN FOR MORE HELPFUL MATERIALS

ADULT POST-RESUSCITATION CARE ALGORITHM

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Immediate treatment

Airway and breathing

- Maintain SpO_2 94 – 98%
- Insert advanced airway
- Use waveform capnography
- Ventilate lungs to normocapnia

Circulation

- 12-lead ECG
- Obtain reliable intravenous access
- Aim for SBP > 100 mmHg:
  - Restore normovolaemia - Fluid (crystalloid)
  - Intra-arterial blood pressure monitoring
  - Consider vasopressor / inotrope to maintain SBP

Prevent fever

Diagnosis

Likely cardiac cause?

NO

YES

ST elevation on ECG and / or electrical or haemodynamic instability?

NO

YES

Coronary angiography ± PCI

Consider head-to-pelvis CT scan

Treat non-cardiac causes and / or life-threatening complications of cardiac arrest

Admit to ICU

Optimising recovery

ICU management

- Prevent fever for at least 72 h
- Maintain normoxia and normocapnia; protective ventilation
- Avoid hypotension; target a MAP > 60-65 mmHg
- Maintain normoglycaemia
- Echocardiography; consider delayed coronary angiography +/- PCI
- Consider mechanical circulatory support if persisting cardiogenic shock
- Diagnose / treat seizures (EEG, sedation, anti-epileptic drugs)
- Delay prognostic judgement for at least 72 h

Secondary prevention

e.g. ICD, screen for inherited disorders, risk factor management

Functional assessments before hospital discharge

Structured follow up after hospital discharge

Rehabilitation

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